THE Open GROUP

TOGAF® Standard – Architecture Requirements Specification

Version 1.0

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Barangay San Antonio Health Department Enterprise Architecture

Barangay San Antonio Health Center, Pasig City

Document Version History

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Tracking Information

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1. Purpose of this Document

The Architecture Requirements Specification provides a set of quantitative statements that outline what an implementation project must do in order to comply with the architecture. An Architecture Requirements Specification will typically form a major component of an implementation contract or contract for more detailed Architecture Definition.

As mentioned above, the Architecture Requirements Specification is a companion to the Architecture Definition Document, with a complementary objective:

The Architecture Definition Document provides a qualitative view of the solution and aims to communicate the intent of the architect.

The Architecture Requirements Specification provides a quantitative view of the solution, stating measurable criteria that must be met during the implementation of the architecture.

1.1 Output and Input

The TOGAF® Content Framework (see Architecture Deliverables) identifies deliverables that are produced as outputs from executing the Architecture Development Method (ADM) cycle and potentially consumed as inputs at other points in the ADM.

This deliverable is produced, updated, and consumed by the ADM Phases:

Deliverable	Output from…	Input to…
Architecture Requirements Specification	B, C, D, E, F, Requirements Management	C, D, Requirements Management

2. Architecture Requirements

The requirements below are measurable statements that implementation projects must satisfy; they derive from the stakeholder requirements R-01 to R-12 in the Architecture Vision.

2.1 Success Measures

ID	Measure	Baseline	Target (proposed)
SM-1	Times a prescription is entered per consultation	Up to 3	1
SM-2	Months with maintenance-medicine stock-outs per year	3 (2026)	0
SM-3	Time to retrieve an old patient's record	2–10 min	< 1 min
SM-4	Monthly reports compiled after working hours	Common	None
SM-5	Hard copies hand-carried to City Hall per month	Every program report	Reduced (City dependent)
SM-6	Staff work files on personal accounts	Most	None
SM-7	Frontline staff (4 aides, 3 BHWs) with computer access	0 of 7	All, shared
SM-8	Late reports	0	Maintain 0

2.2 Business Service Contracts

Service	Service level today	Requirement
Consultation	Mon/Tue/Fri 8–11 AM; 360 last month	Numbered queue; first-time patients registered with consent in one step
Maintenance medicine	Mon/Wed/Fri 1–4 PM; one-month supply; 297 pickups last month	No stock-outs; quantity limits only in declared shortages
Immunization	Wednesdays; MR-SIA campaigns	Vaccine fridge temperature checked and logged daily
Dental	Tue–Thu, scheduled	Appointments honored within slot
Program reporting	Before the first Wednesday monthly	On time, within working hours
Referral	About 30 a month	Every referral logged with destination and outcome

2.3 Application Service Contracts

- The City EMR remains available for registration, diagnosis and prescriptions (City-owned; no change by the center).
- A barangay-owned shared workspace provides storage for program files, including OPT files of 2–3 GB per cycle.
- A stock-tracking sheet and a tally sheet are usable by aides and BHWs on a shared computer.
- Paper fallback procedures cover outages (about 5 a month).

2.4 Implementation Guidelines

- Apply the three principles: One Patient, One Record; Use the City Record, Don't Rebuild It; Report Once, Count Many Times.

- Prefer simple, low-cost tools the barangay can fund and support.
- Keep paper fallback for outages.
- Roll out outside January–March and month-end reporting.

2.5 Implementation Specifications

Specified per work package in the Architecture Roadmap and the Architecture and Solution Building Blocks (Phase E).

2.6 Implementation Standards

- Data Privacy Act of 2012 (RA 10173): consent on ITR and charts; restricted access to mental health records (nurse, midwife and doctor only).
- DOH record retention: 7 years, with logged disposal (Records Storage/Retention/Disposal logbook).
- PhilHealth YAKAP / GAMOT rules for registration, prescriptions (UPSC, QR code) and claims.
- DOH/NNC guidelines: FHSIS, PPAN, First 1000 Days, OPT Plus validity of at least 80%.

2.7 Interoperability Requirements

- Prescriptions entered in the City EMR should be usable by PhilHealth YAKAP without retyping (request to CHD).
- EMR reports must be exportable to spreadsheets (already available, e.g., morbidity report) for program reports.
- Dental records should flow between dental eMED and the EMR.

2.8 IT Service Management Requirements

- Named support contact for barangay-owned devices (barangay IT team); City vendor for the EMR.
- Backup internet connection for clinic hours.
- Device inventory kept by the custodian (Ms. Josefina G. Roa); City audit about every 6 months; barangay check every 3 months.

2.9 Constraints

- City and national systems cannot be changed by the center.
- Minor process or system changes need City Health Officer approval; major ones need the City Mayor.
- Physician, dentist, nurse, midwife, nutritionist, encoder and BNS are permanent City staff; Pasig Health Aides are City job order; BHWs are barangay contractual.
- Medicines and dental supplies come through City procurement and bidding.
- Barangay funds follow the annual budget; 2027 proposals are due November 2026.

- No patient-level data may be used in this project.

2.10 Assumptions

- Interview figures (budgets, volumes) are approximate and self-reported unless verified on October 22, 2026.
- Barangay leadership will consider costed items in the 2027 budget.
- Staffing levels remain the same in 2027.
- The City EMR remains the system of record.

A Abbreviations

Abbreviations, acronyms and reference codes used in this document and its figures, in alphabetical order.

Abbreviation	Meaning
ADM	Architecture Development Method, the step-by-step method of the TOGAF Standard
BHW	Barangay Health Worker
BNS	Barangay Nutrition Scholar
CHD	City Health Department (Pasig City)
DOH	Department of Health
eMed / eMED	Name of the City's electronic medical record system; “dental eMED” is its separate dental system
EMR	Electronic Medical Record
FHSIS	Field Health Services Information System (DOH)
GAMOT	PhilHealth Guaranteed and Accessible Medications for Outpatient Treatment
GB	Gigabyte (a unit of file size)
Hon.	Honorable (title of elected officials)
IT	Information technology
ITR	Individual Treatment Record (the patient's registration form)
MR-SIA	Measles-Rubella Supplemental Immunization Activity
NNC	National Nutrition Council
OPT / OPT Plus	Operation Timbang Plus: yearly weighing and measuring of children aged 0–59 months
PhilHealth	Philippine Health Insurance Corporation
PPAN	Philippine Plan of Action for Nutrition
QR	Quick Response (a scannable square barcode)
R-01 …	Stakeholder requirement reference number
RA	Republic Act (a Philippine law)
SM-1 …	Success measure reference number
TOGAF	The Open Group Architecture Framework (the TOGAF® Standard)
UA&P	University of Asia and the Pacific
UPSC	Unique Prescription Security Code
YAKAP	PhilHealth Yaman ng Kalusugan Program (primary-care benefit)